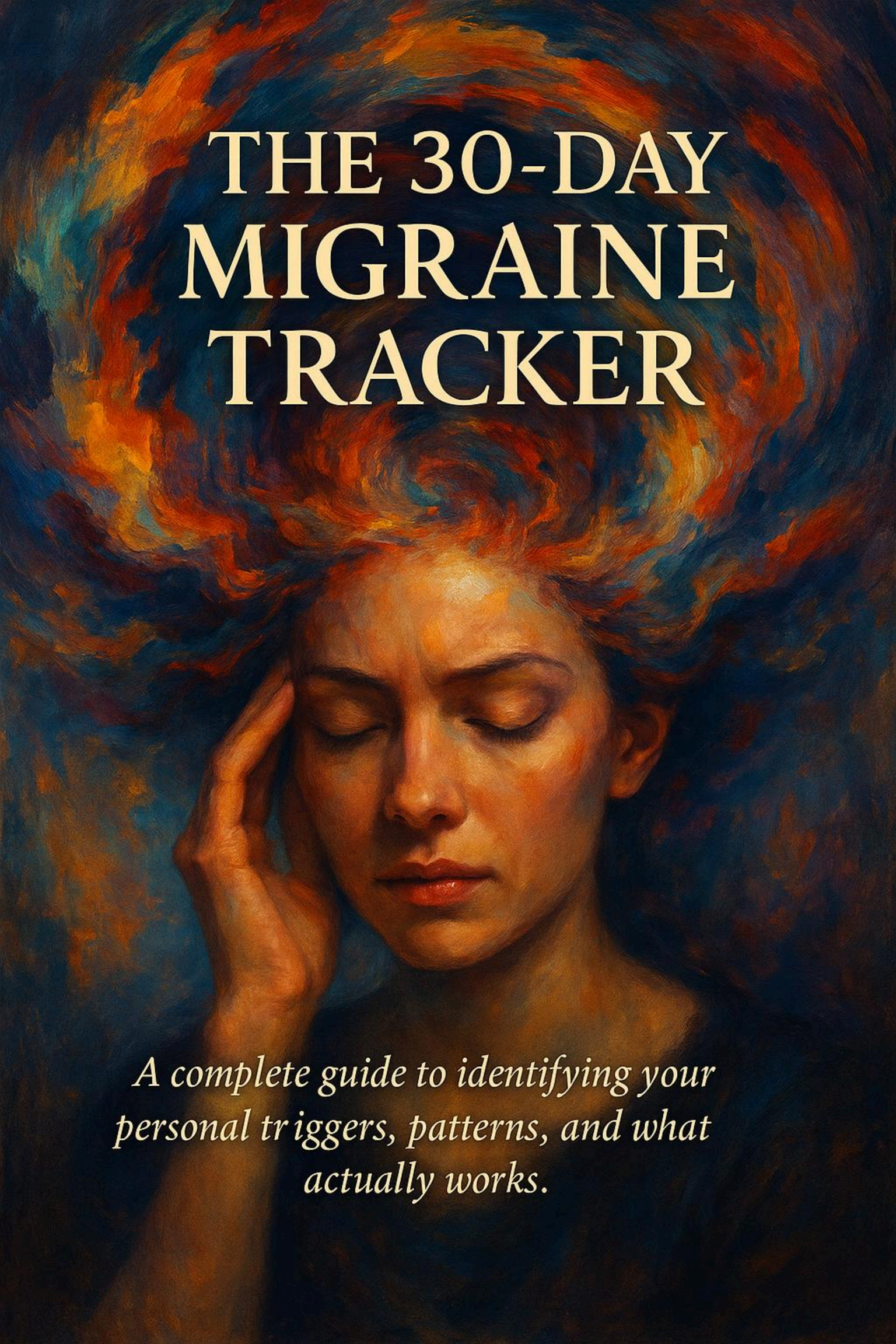


The background of the entire image is a painting of a woman's face and upper torso. She has her eyes closed and a pained expression, with her right hand pressed against her forehead. Surrounding her head is a large, turbulent, circular halo of swirling colors, primarily deep blues, oranges, and yellows, resembling a storm or a fire. The text is overlaid on the upper portion of this image.

THE 30-DAY MIGRAINE TRACKER

*A complete guide to identifying your
personal triggers, patterns, and what
actually works.*

A NOTE BEFORE YOU BEGIN

Read this first.

This guide is intended for informational and educational purposes only. It is not medical advice. Migraine is a complex neurological condition. The strategies described in this document may support your overall wellbeing but are not a substitute for diagnosis, treatment, or care from a qualified healthcare professional.

Always consult your physician before making changes to your medication, diet, or treatment plan.

Cutaneous allodynia, central sensitization, and other clinical terms referenced in this guide are widely documented in peer-reviewed neurological literature. References are listed on page 27.

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WELCOME

If your hair has ever hurt.

If you've ever had to cancel plans because your scalp couldn't bear a hat. If a strand of hair brushing your forehead felt like a knife. If your doctor handed you a prescription with a shrug and said "try this" — this guide is for you.

We don't promise to cure your migraines. No book can do that. And anyone who claims otherwise is selling you something built on hope instead of evidence.

What we will do is hand you a system. Thirty days of structured observation. Twenty-seven techniques that chronic migraine sufferers across the UK and US have used to reduce attack severity. And a clear explanation of cutaneous allodynia — the symptom that's invalidated as often as it's experienced.

You are not crazy. You are not exaggerating. You are dealing with a serious neurological event that the world is slow to take seriously.

— *The Kalmely team*

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PART ONE

Cutaneous Allodynia Explained.

When your hair hurts.

92%

OF CHRONIC MIGRAINEURS
EXPERIENCE CUTANEOUS ALLODYNIA

What is cutaneous allodynia.

In approximately 80% of episodic migraine sufferers and over 92% of chronic migraine sufferers, attacks bring with them a strange and isolating symptom: the sensation that ordinary touch has become painful.

A hair tie that felt fine an hour ago now triggers shooting pain across your scalp. The seam of a pillow case feels rough enough to leave a mark. Wearing a hat becomes impossible.

This is **cutaneous allodynia** — a real, documented neurological phenomenon. A measurable clinical state in which the central nervous system amplifies normal tactile input into pain.

Why it happens.

During a migraine attack, neurons in the **trigeminal nucleus caudalis** — the part of the brainstem that receives sensory input from your face and scalp — become hyper-excitabile.

This is called **central sensitization**. Once it's switched on, nerve fibres that normally carry signals about light touch (A-beta fibres) are misinterpreted by your brain as pain signals.

"You aren't imagining it. Your nervous system is, quite literally, misreading reality."

This explains why a hair tie, a pillowcase seam, or a tight hat — innocuous touches at any other time — can become genuinely painful during an attack.

Three things to know.

O1

It's a marker of disease progression.

Patients with cutaneous allodynia are approximately 2.5 times more likely to transition from episodic to chronic migraine. Tracking it matters.

O2

It changes what relief options work.

Standard advice — ice packs, weighted masks, head wraps — often makes allodynia worse during peak attacks. The right tool depends on which phase you're in.

O3

It is invalidated more than any other symptom.

Because it's invisible, intermittent, and sounds strange, sufferers are routinely told they're dramatic. This is not your fault. The literature has documented it since the 1990s.

When you fill in your tracker, note specifically when your scalp, forehead, or hair feels tender or painful. Patterns will emerge. They almost always do.

PART TWO

The 30-Day Migraine Tracker.

A simple daily log for finding your patterns.

How to use this tracker.

The goal is not to obsess. The goal is to build a month of honest, structured data so you can see what your body is actually doing — not what you remember it doing.

Five minutes a day. Same time, ideally evening.

Each daily page contains a severity scale (0–10), a symptom checklist (including allodynia indicators), a trigger checklist, sleep tracking, and a small notes area.

After 30 days, you'll spot:

- Which days of the week tend to be worse
- Whether sleep deficit predicts attacks
- Whether allodynia precedes or follows the headache
- Which foods, hormones, weather, or stress cluster with attacks
- What works to shorten or soften your attacks

Bring the completed tracker to your next neurology appointment. A 30-day log changes the quality of that conversation significantly.

The severity scale.

Use this 0–10 scale consistently across all 30 days. Score the *peak* severity of your day, not the average.

0	No pain, no symptoms.	6	Significant. I want to lie down. Light is hard.
1	Barely there. Aware of slight pressure.	7	Severe. I can't work normally. Dark quiet room.
2	Mild. I can fully function.	8	Severe + allodynia. Hair, skin, scalp painful.
3	Mild but distracting. I notice it.	9	Severe + nausea + vomiting. Bedridden.
4	Moderate. I'd take an OTC analgesic if I could.	10	Worst attack of my life. I'd go to A&E.
5	Moderate. I'm slower. Light hurts a bit.		

Common migraine triggers.

Tick any that applied in the 24 hours before an attack. Use this as a prompt when filling in each daily log.

HORMONAL

- ☐ Menstrual cycle phase
- ☐ Ovulation
- ☐ Perimenopause symptoms
- ☐ Hormonal contraceptive cycle

EMOTIONAL

- ☐ High-stress day
- ☐ Stress letdown (day after)
- ☐ Argument / emotional difficulty
- ☐ Sleep anxiety

ENVIRONMENT

- ☐ Bright or flickering light
- ☐ Sun glare
- ☐ Loud noise
- ☐ Strong scents
- ☐ Barometric pressure change
- ☐ Storm approaching
- ☐ Heat / humidity spike

PHYSICAL

- ☐ Poor sleep / <6 hrs
- ☐ Slept >9 hrs
- ☐ Skipped exercise window
- ☐ Intense exercise
- ☐ Dehydration
- ☐ Eye strain (screens)
- ☐ Neck / jaw tension

FOOD & DRINK

- ☐ Skipped a meal
- ☐ Caffeine excess / withdrawal
- ☐ Alcohol (esp. red wine, dark spirits)
- ☐ Aged cheese
- ☐ Processed meats / nitrates
- ☐ MSG / glutamate-rich foods
- ☐ Artificial sweeteners
- ☐ Citrus
- ☐ Chocolate

Daily tracker.

Print this page **30 times**. Or duplicate it digitally. Fill one in each evening for 30 days, even on attack-free days — the blank days are data too. *Pro tip: keep it on your nightstand and fill it in before bed.*

Daily Log

DATE: _____ DAY ____ / 30

SEVERITY 0-10

SLEEP HRS

QUALITY 1-5

PAIN LOCATION

forehead · L temple · R temple · crown · back · neck · eye · all-over

ATTACK PHASE

none · prodrome · aura · attack · postdrome

☐ Throbbing

☐ Sound sensitive

☐ Visual aura

☐ Hair hurts →

☐ Stabbing

☐ Smell sensitive

☐ Brain fog

☐ Skin painful →

☐ Pressure / vise

☐ Nausea

☐ Neck stiffness

☐ (arrows = allodynia indicators)

☐ Light sensitive

☐ Vomiting

☐ Scalp tender →

TRIGGERS NOTED

INTERVENTIONS

WHAT HELPED MOST

WHAT MADE IT WORSE

PART THREE

27 Drug-Free Tactics.

For in-attack relief.

TACTICS 01–05

Sensory deprivation.

The single most reliable approach for many migraineurs: remove as much sensory input as possible. Light, sound, smell, and touch all amplify central sensitization once an attack is underway.

01 Total blackout.

Heavy curtains, eye mask, blanket over the head — the goal is zero light reaching the retina. Even a sliver through curtain edges can sustain the attack.

02 Earplugs + noise-cancelling headphones.

Don't choose between them. Layer both. The ambient sound of traffic or household noise that doesn't normally register can be unbearable during attacks.

03 Remove fragrances from the room.

Osmophobia (smell sensitivity) affects up to 70% of migraineurs. Remove perfume, candles, diffusers, and plug-ins from your sleeping space permanently.

04 Cool, neutral-temperature room.

Aim for 17–19°C. Hot rooms exacerbate vasodilation. Cold rooms can trigger cold-shock responses in those prone to it.

05 Phone face-down, screens off.

Even the glow of a notification can pull you out of sensory recovery. Use airplane mode during the worst hours.

TACTICS 06–13

Pressure & temperature.

06 Gentle pressure to temples.

For non-allodynic attacks, gentle temple pressure disrupts pain signals via the Gate Control mechanism. The key word is *gentle*.

07 Avoid heavy ice packs with scalp allodynia.

Counterintuitive but critical. Hard ice packs trigger cold shock, spike cortisol, and aggravate sensitized nerves.

08 Warm — not hot — compress on the neck.

A 35–45°C flannel on the back of the neck relaxes the trapezius. Avoid above 50°C — too hot triggers vasodilation.

09 Rhythmic compression therapy.

Devices that deliver rhythmic, low-pressure compression across multiple zones ease muscular co-spasm without triggering allodynia. (*Kalmely's mechanism.*)

10 Soft elastic forehead band.

A soft elastic band low on the forehead provides proprioceptive input. Avoid tight bands or hard fasteners.

11 Acupressure: LI-4 (Hegu).

Back of the hand between thumb and index finger. Press firmly for 1–2 minutes. Avoid during pregnancy.

12 Acupressure: GB-20 (Fengchi).

Base of the skull, in the hollows beside the spine. Gentle thumb pressure for 1–2 minutes eases occipital tension.

13 Skull base release with a tennis ball.

Lying down, place a soft tennis ball under the base of your skull. Let gravity do the work. 5–10 minutes maximum.

TACTICS 14 – 18

Hydration & nutrition.

I4 Electrolyte solution at attack onset.

Plain water is rarely enough during an attack. Dehydration is often coupled with low sodium. A balanced electrolyte drink (sodium + potassium + magnesium) sipped slowly can interrupt mild attacks.

I5 Coffee — once, early, with food.

For some, a single early-day coffee with food reduces attack severity by ~30% via vasoconstriction. For others, it triggers next-day withdrawal attacks. Track to find your group.

I6 Ginger tea (fresh, not bagged).

Strong evidence supports ginger as anti-nausea during attacks. Slice fresh ginger, steep 10 minutes, drink warm.

I7 Magnesium glycinate, consistently.

600mg daily of magnesium glycinate has been shown to reduce migraine frequency by ~20% in deficient individuals. Glycinate is better absorbed than oxide. Discuss with your physician first.

I8 Eliminate suspected food triggers one at a time.

Don't eliminate everything at once. Use your tracker to identify a suspected trigger (red wine, aged cheddar, MSG-heavy takeaways), then remove just that one for 4 weeks. Compare frequency.

TACTICS 19 – 23

Breathing & grounding.

19 4-7-8 breath.

Inhale through nose 4 counts. Hold 7. Exhale through pursed lips 8. Repeat 4 cycles. Activates the parasympathetic nervous system and eases autonomic activation.

20 Box breathing.

4-4-4-4. Inhale 4, hold 4, exhale 4, hold 4. Used by Navy SEALs to manage stress. Easier than 4-7-8 if your breathing is already shallow.

**21 Cold-water-on-wrists
grounding.**

Run cold water over inner wrists for 30 seconds. The vagus nerve responds to cold by signalling "rest and recover" to the brain.

22 Bilateral self-tapping.

Lightly alternate-tap your knees or shoulders for 2 minutes. Used in trauma therapy (EMDR) — interrupts rumination loops during anxious attacks.

23 The 5-4-3-2-1 sensory anchor.

Name 5 things you see, 4 you touch, 3 you hear, 2 you smell, 1 you taste. For migraine, do it with eyes closed (skip the seeing) to avoid photophobia trigger.

TACTICS 24 – 27

Sleep optimization.

24 Same bedtime, same wake time.

Migraine brains are unusually sensitive to circadian disruption. Even on weekends, hold your sleep schedule within a 30-minute window. Travel and shift work are common triggers.

25 Blackout, cool, quiet.

The same conditions that abort an attack also prevent one. 17–19°C, complete darkness, no standby lights, no street light through curtains.

26 Wind-down protocol — 60 min before bed.

No screens. No work. Warm shower. Dim lights to 30% or below. Same routine every night. Your brain learns the pattern and prepares for sleep faster.

27 Wake without startling.

A jolt-awake from a loud alarm activates the sympathetic nervous system and can trigger morning attacks. Use a gradual sunrise alarm or soft vibrating watch alarm instead.

PART FOUR

Putting it together.

After 30 days, the patterns become visible.

After 30 days, look for:

01 Day-of-week clusters — Tuesday-after-Monday-stress is real.

02 Sleep deficit thresholds — most chronic migraineurs have an attack within 48h of any night below 6 hours.

03 Hormonal phase clusters — for those who menstruate, day -2 through day +2 is the most common attack window.

04 Weather pattern clusters — low-pressure days incoming = pre-emptive recovery day.

05 Specific food triggers that cluster within 24h of attacks.

You are building a custom protocol for your nervous system. No one else can do this for you. But you can do it for yourself — with five minutes a day and a month of attention.

— The Kalmely team

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The cutaneous allodynia statistics and central sensitization mechanism described in this guide are drawn from peer-reviewed neurological literature.

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ABOUT

A note on Kalmely.

This guide is yours to keep regardless of whether you ever buy our product. We mean that. The tracker, the tactics, and the allodynia explainer were written to stand on their own.

That said, if you've found that ice caps worsen your allodynia, that fixed-weight masks are intolerable during peak attacks, or that you've been searching for a way to apply gentle, rhythmic compression with adjustable heat — Kalmely was designed exactly for you.

Six independently controllable airbags. Three heat levels (35–55°C). Four massage modes. Wireless. 15-minute sessions. £119 in the UK. Free 30-day return.

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who deserve more than a prescription pad.*

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